Lena Whitmore

PRACTICE EXAM 17 / EXAM SET 03

Domain 3: Clinical Patient Care

84 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Infection Control and Safety

> **SCENARIO** During a task requiring a medical mask, the assistant's mask becomes wet and loses its normal fit. The clinic procedure requires replacement of wet masks using the appropriate removal and hand-hygiene steps.

What should the assistant do?

A. Turn the wet mask inside out and continue wearing it.

B. Keep the mask for the rest of the shift because it was new that morning.

C. Replace the wet mask using the specified removal and hygiene procedure.

D. Place the wet mask in a pocket to dry between patients.

Q02 | Phlebotomy

> **SCENARIO** An adult needs finger capillary collection. The supplied collection guide recommends the lateral fleshy pad of the middle or ring finger and excludes visibly injured or heavily callused sites. An uninjured ring-finger lateral pad is available.

Which site follows the guide?

A. The uninjured lateral fleshy pad of the ring finger.

B. A heavily callused thumb pad.

C. A visibly swollen, injured index-finger tip.

D. The center of the palm between major creases.

Q03 | Infection Control and Safety

SCENARIO A multidose medication vial is taken into an immediate patient-treatment area. The clinic policy, consistent with its injection-safety procedure, requires vials entering that area to be dedicated to that patient rather than returned to a shared supply.

What should happen to this vial?

A. Remove the patient designation and use it for anyone on the same day.
B. Combine its remainder with another shared vial.
C. Keep the vial patient-dedicated or dispose of it under the stated policy.
D. Return it to shared stock because the rubber stopper looks clean.

Q04 | Phlebotomy

SCENARIO A neonatal capillary collection order is being prepared. The clinic's approved device sheet sets a maximum puncture depth of 1.5 mm for this patient and lists an age-approved, fixed-depth 1.2 mm safety lancet as available. Other available devices are fixed at 1.8 mm and 2.4 mm or have unreadable depth markings.

Which device meets the supplied criteria?

A. The age-approved fixed-depth 1.2 mm safety lancet.
B. The device with unreadable depth markings.
C. The fixed-depth 1.8 mm device.
D. The fixed-depth 2.4 mm device.

Q05 | Point of Care Testing and Laboratory Procedures

SCENARIO A specified CLIA-waived cartridge assay loses power midway through its run. Its instructions say an interrupted run cannot be reported and must be repeated on a new cartridge after the system is restored. No valid result is displayed.

How should the assistant handle the test?

A. Restart the same used cartridge even though the instructions require a new one.
B. Estimate the result from the patient's symptoms.
C. Follow the interruption procedure and repeat with a new cartridge after restoration.
D. Report the partially displayed startup number as the patient result.

Q06 | General Patient Care

SCENARIO Two clear, unlabeled syringes remain on a medication-preparation tray after an interruption. No one can reliably establish their contents, and neither has been administered. Clinic policy prohibits administration of medication whose identity is uncertain.

How should the assistant handle them?

A. Retain the syringes for administration after another person guesses their identity.
B. Remove them from use and arrange safe disposal and verified replacement under policy.
C. Use the tray positions to decide which clear fluid corresponds to each order.
D. Assign syringe labels according to the expected preparation order without verifying the contents.

Q07 | Phlebotomy

SCENARIO A routine venous collection is complete, and the needle is still in the vein. The clinic removal procedure requires tourniquet release before needle withdrawal. The tourniquet is still applied.

Which step is needed before withdrawal?

A. Ask the patient to bend the elbow around the inserted needle.
B. Release the tourniquet as specified while maintaining safe needle control.
C. Tighten the tourniquet further to hold the vein still.
D. Apply an adhesive bandage over the needle while it remains inserted.

Q08 | Point of Care Testing and Laboratory Procedures

SCENARIO During provider-directed skin allergy testing, the supplied protocol says a raised wheal of 3 mm or more at the negative-control site requires clinician review before any allergen findings are interpreted. The assistant measures a 4 mm negative-control wheal.

Which reporting action is appropriate?

A. Tell the patient the control alone establishes a confirmed allergy diagnosis.
B. Subtract the control size from allergen measurements without an authorized interpretation.
C. Ignore the raised control and report all allergen sites using the usual interpretation.
D. Record and flag the negative-control finding for clinician review without assigning allergy diagnoses.

Q09 | General Patient Care

> **SCENARIO** A provider requests semi-Fowler positioning for a comfortable patient during an examination. The order specifies a head-of-bed elevation of 30-45 degrees, and the examination table adjusts safely.

Which setup follows the order?

A. Raise the table height while leaving the backrest flat.
B. Elevate the head of the table to the specified range and support the patient safely.
C. Place the patient face down with the head below the feet.
D. Position the patient completely flat without head elevation.

Q10 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A patient reports ear pain before a planned hearing screen. The clinic screening procedure requires clinician review of new ear pain before proceeding; the assistant is not authorized to diagnose or remove an obstruction.

What should the assistant do?

A. Record a failed hearing diagnosis without performing the screen.
B. Remove any suspected obstruction with an available instrument.
C. Increase the tone intensity until the painful ear responds.
D. Report the pain and obtain the required clinician review before screening.

Q11 | Patient Intake and Vitals

> **SCENARIO** A patient reports taking one tablet each morning, but the current medication list says one tablet twice daily. The bottle brought to the visit is unavailable, and the patient is comfortable.

What should the assistant record during intake?

A. Replace the report with twice-daily use because that is already in the chart.
B. Remove the medication from the list because the directions do not match.
C. Tell the patient to resume twice-daily use before the provider reviews it.
D. Record the actual reported use and flag the difference from the listed directions for clinician review.

Q12 | General Patient Care

Where is medication placed for the buccal route?

A. Between the cheek and gum.
B. On top of the tongue for immediate swallowing.
C. Under the tongue.
D. Against the skin behind the ear.

Q13 | General Patient Care

> **SCENARIO** An assistant trained and authorized for oral medication administration has a clinically verified order for 75 mcg once. The verified tablets contain 0.05 mg each and are approved for splitting into equal halves. Use 1 mg = 1,000 mcg.

How many tablets provide the ordered dose?

A. 1.5 tablets
B. 15 tablets
C. 0.15 tablet
D. 0.5 tablet

Q14 | Patient Intake and Vitals

> **SCENARIO** A child's measured height is 4 ft 6 in. The form requires centimeters to one decimal place. Use 12 in per foot and 2.54 cm per inch.

Which converted height is correct?

A. 114.3 cm
B. 137.2 cm
C. 152.4 cm
D. 121.9 cm

Q15 | EKG and Cardiovascular Testing

At a supplied EKG scale of 0.04 second per small horizontal box, what duration is represented by 8 small boxes?

A. 0.32 second
B. 0.20 second
C. 0.40 second
D. 3.2 seconds

Q16 | General Patient Care

> **SCENARIO** An assistant discovers that a medication dose already given was larger than the verified order. The patient is currently alert, and the clinician is immediately available. Clinic policy requires prompt clinical notification with the actual drug, dose, route, and time.

What should the assistant do immediately?

A. Observe until the next routine check and report only if symptoms develop.
B. Ask the patient to decide whether the larger dose needs clinical review.
C. Document the expected ordered dose while awaiting a chance to speak to the clinician.
D. Notify the clinician with the actual administration details and follow the directed patient-care response.

Q17 | Point of Care Testing and Laboratory Procedures

SCENARIO An employment-related specimen collection uses a supplied chain-of-custody procedure requiring a tamper-evident seal, the collector's entries, and documented handoff. The assistant has collected the specimen but has not yet transferred it.

Which action maintains the specified collection record?

A. Send an unsealed cup and complete the collector entries from memory next week.
B. Have the courier sign for an earlier time before the specimen is ready.
C. Complete the seal and collection entries and document each handoff as required.
D. Remove the custody form so the laboratory cannot see who collected it.

Q18 | Infection Control and Safety

SCENARIO While handling a potentially contaminated specimen rack, an assistant notices a tear in one examination glove. No skin injury or visible skin contamination is apparent. The clinic procedure requires changing damaged gloves with hand hygiene and evaluating any actual exposure.

What should the assistant do?

A. Finish handling clean supplies before replacing the damaged glove.
B. Stop the task safely, remove the damaged gloves, perform hand hygiene, and replace the gloves.
C. Put a second glove over the torn glove and continue without hand hygiene.
D. Cover the tear with tape while the glove remains on the hand.

Q19 | Phlebotomy

SCENARIO During a routine venous draw, the first tube fills normally. The next compatible tube draws no blood, although the patient is comfortable and the needle position has not changed. The collection instructions direct trying a new compatible tube for suspected loss of tube vacuum before further action.

What should the assistant do?

A. Transfer blood from the filled tube into the nonfilling tube to meet its volume.
B. Try a new compatible tube as instructed while maintaining safe needle control.
C. Continue waiting with the nonfilling tube without following the specified replacement step.
D. Redirect the needle repeatedly before checking a possible tube-vacuum problem.

Q20 | Infection Control and Safety

SCENARIO A device's high-level disinfection instructions require three separate rinses with fresh specified-quality water before patient use. Only one rinse has been completed.

Which action follows the instructions?

A. Use the device after the first rinse because the soak time was adequate.
B. Dry the device without further rinsing to remove chemical residue.
C. Repeat rinsing in the same used water and count it as three separate rinses.
D. Complete the remaining required fresh-water rinses before further preparation or use.

Q21 | General Patient Care

SCENARIO An assistant must add a sterile gauze package to an established sterile field. The package is intact, and the clinic technique permits opening the wrapper so the contents drop onto the field without the wrapper or hands touching it.

Which action follows the technique?

A. Reach into the package with an ungloved hand to position the gauze.
B. Open the package without touching its contents and let the gauze fall onto the sterile field.
C. Place the outer wrapper flat on the field before opening it.
D. Drag the wrapper across the field to slide out its contents.

Q22 | Point of Care Testing and Laboratory Procedures

SCENARIO On a specified CLIA-waived system, required QC failed twice. After troubleshooting, a third run passes. The written clinic process requires retaining every QC result and supervisor review of repeated failures before patient testing resumes. Patient testing is still paused.

What should the assistant do next?

A. Resume on the single passing run without the required repeated-failure review.
B. Retain the runs but omit the required review because the last run passed.
C. Record all three runs and obtain the required review before resuming testing.
D. Retain only the passing run and leave the failed runs out of the QC record.

Q23 | General Patient Care

SCENARIO A provider-approved plan requires non-weight-bearing on the right leg during an assisted transfer. The assistant has been trained in the specified technique, and the required helper and equipment are ready.

Which movement preserves the stated weight-bearing restriction?

A. Push through the right foot briefly to initiate standing.
B. Keep the right foot clear of the floor during the planned assisted maneuver.
C. Use the right toes for weight-bearing support while turning.
D. Place most body weight on the right heel during the pivot.

Q24 | Phlebotomy

> **SCENARIO** During venipuncture, a patient becomes unresponsive and starts to slump. The collection chair supports reclining, a trained coworker is beside the assistant, and clinic procedure calls for stopping the draw, securing sharps, supporting the patient, and activating the response team.

Which response follows the procedure?

A. Finish collecting the remaining tubes before removing the needle and calling for help.

B. Stop the draw, safely remove and secure the needle while help supports the patient, and activate the response.

C. Support the patient but delay emergency activation until the patient awakens.

D. Call for help but leave the inserted needle unsecured while the patient slumps.

Q25 | EKG and Cardiovascular Testing

> **SCENARIO** A stable patient wearing an ambulatory recorder reports that one electrode has detached. The device instructions provide a labeled placement map, a matching spare electrode, and directions to note the detachment and reattachment times; unresolved signal problems require a call to the clinic.

Which instruction follows this device procedure?

A. Place the spare anywhere on the chest and omit the interruption from the diary.

B. Remove every remaining electrode and wait until the scheduled return visit.

C. Erase earlier recordings so the gap will not appear in the record.

D. Follow the mapped replacement and time-recording instructions; call if the signal remains faulty.

Q26 | General Patient Care

> **SCENARIO** An ordered oral liquid must be measured and delivered by a trained, authorized assistant. The clinic supplies a dedicated oral syringe with appropriate volume markings and a separate parenteral syringe.

Which device should the assistant select for the oral task?

A. The parenteral syringe with an injection needle attached.

B. An unmarked specimen-transfer dropper.

C. The appropriately marked dedicated oral syringe.

D. A household eating spoon of unknown capacity.

Q27 | General Patient Care

> **SCENARIO** An assistant is performing an ordered dressing change. The old dressing adheres to the wound. The approved procedure directs moistening an adherent dressing with sterile saline and allowing it to loosen before gentle removal.

Which action follows the procedure?

A. Scrape beneath the adherent area with a sharp blade.
B. Moisten with sterile saline and wait for the adherent dressing to loosen before removal.
C. Pour surface disinfectant onto the dressing to dissolve it.
D. Pull the dry dressing off rapidly to shorten discomfort.

Q28 | Infection Control and Safety

> **SCENARIO** A procedure counter has visible blood on it. The approved product instructions require removal of visible soil followed by the labeled disinfectant process. A coworker proposes a quick disinfectant pass over the blood without cleaning it away.

Which approach follows the instructions?

A. Disinfect only the clean area around the visible blood.
B. Cover the blood with paper and consider the surface ready for clean supplies.
C. Remove the visible soil using the specified precautions, then complete the labeled disinfection process.
D. Leave the blood in place and shorten contact time because the product is strong.

Q29 | Patient Intake and Vitals

> **SCENARIO** For waist measurement, the clinic technique specifies a level tape at the stated iliac-crest landmark, snug without compressing skin, at the end of a normal exhalation. The patient holds the abdomen inward and holds the breath.

Which instruction best corrects the patient's preparation?

A. Keep the breath held and press the tape firmly into the skin.
B. Take the deepest possible breath and hold it while the tape is tightened.
C. Pull the abdomen inward more tightly and keep holding the breath.
D. Relax the abdomen and breathe normally so the reading can be taken at normal exhalation.

Q30 | EKG and Cardiovascular Testing

SCENARIO A resting EKG channel drops out intermittently despite appropriate skin preparation and fresh electrodes. During the manufacturer-approved cable self-test, the same fault occurs with one cable but not with its approved replacement. The patient is stable.

Which action is appropriate before acquisition?

A. Remove the faulty cable from use and verify the approved replacement through the equipment process.
B. Repeat skin preparation without addressing the isolated cable fault.
C. Keep the failed cable and record the demonstrated dropout as a patient finding.
D. Acquire another patient recording with the failed cable before following the replacement process.

Q31 | Infection Control and Safety

SCENARIO Before a skin procedure, an assistant finds a wipe labeled for hard environmental surfaces only. The selected wipe is not labeled as a skin antiseptic, and the clinic has an approved skin-preparation product.

Which product should be used for the skin-preparation task?

A. Whichever product has the strongest odor.
B. A mixture of the two products to combine their effects.
C. The approved skin antiseptic appropriate to the procedure.
D. The environmental-surface wipe because a disinfectant must be suitable for skin.

Q32 | General Patient Care

SCENARIO The EHR shows two active orders for the same nonurgent procedure on the same patient and day. Both appear to have been entered from one provider request. Clinic workflow requires confirmation of duplicate orders before performance.

What should the assistant do?

A. Clarify the intended procedure order and reconcile the duplicate through the approved workflow.
B. Choose whichever order appears first and leave the other marked completed.
C. Perform the procedure twice because two active orders are visible.
D. Delete both orders without obtaining confirmation.

Q33 | Patient Intake and Vitals

> **SCENARIO** A patient leaves one required item on an intake screening form blank. The scoring instructions state that unanswered items must not be treated as negative responses and that incomplete forms require clinician review.

How should the assistant handle the form?

A. Score the blank item as negative so a total can be generated.

B. Mark the form complete because most items were answered.

C. Identify it as incomplete and follow the review process without assigning a negative answer.

D. Copy the answer from a different patient of the same age.

Q34 | General Patient Care

> **SCENARIO** Immediately before a planned skin procedure, the patient points to a different lesion from the one named in the verified procedure plan. The provider has not yet begun, and the assistant cannot determine the intended lesion independently.

What should the assistant do?

A. Pause preparation and have the patient and provider resolve the intended target before proceeding.

B. Prepare both lesions and assume both should be treated.

C. Ignore the statement because the schedule was printed earlier.

D. Select the lesion that looks most abnormal to the assistant.

Q35 | General Patient Care

> **SCENARIO** A vaccine manufacturer issues a notice about a specific lot. The clinic must identify which patients received that lot, and the administration records include drug, site, date, manufacturer, and lot information.

Which recorded information most directly supports this search?

A. The name of the patient's usual pharmacy alone.

B. The patient's room number and the clinic's opening time.

C. The product manufacturer and lot number linked to each administration.

D. The injection site alone, regardless of product details.

Q36 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** The throat-swab collection instructions for an ordered test specify sampling the indicated posterior throat area while avoiding the tongue and cheeks. Before reaching that area, the swab rubs the tongue. The instructions direct a new swab after such contact.

What should the assistant do?

A. Use the touched swab because any mouth sample is equivalent.
B. Use a new swab and repeat the specified collection without the unintended contact.
C. Label the collection as posterior throat without repeating it.
D. Wipe the swab on dry gauze and then sample the throat.

Q37 | Infection Control and Safety

> **SCENARIO** An open formalin container spills in a specimen area, and staff notice strong vapors. The assistant has not been trained for chemical-spill cleanup. The posted plan requires leaving the affected area, warning others, and contacting the designated trained response team.

Which action should the assistant take?

A. Attempt a small cleanup first despite lacking the required spill-response training.
B. Remain beside the spill while asking a coworker to find an improvised absorbent.
C. Leave the affected area, warn others, and activate the designated trained response.
D. Delay warning others until the assistant has completed a detailed incident note.

Q38 | General Patient Care

> **SCENARIO** An assistant is trained and authorized to give prescribed ear drops to an adult. The supplied product procedure directs gentle upward-and-backward traction on the pinna before instillation.

Which positioning action follows the procedure?

A. Gently pull the pinna upward and backward before instilling the ordered drops.
B. Leave the patient standing and direct the drops against the outside of the ear.
C. Insert the applicator deeply until it touches the eardrum.
D. Pull the pinna downward and forward in place of the specified direction.

Q39 | EKG and Cardiovascular Testing

> **SCENARIO** During a provider-supervised exercise EKG, the patient asks to stop because of worsening knee pain. The clinic test procedure requires promptly alerting the supervising clinician and qualified treadmill operator when a patient requests termination. The assistant is not authorized to independently alter the exercise protocol.

What should the assistant do?

A. Tell the patient to continue until the preset duration ends regardless of the request.
B. Increase the treadmill setting independently to finish the target workload sooner.
C. Document the request after the visit without telling the testing team.
D. Immediately relay the stop request to the supervising team and follow the test-termination procedure.

Q40 | Infection Control and Safety

> **SCENARIO** A portable oxygen cylinder is standing unsecured beside the examination table. The equipment instruction requires storage or movement in its approved secured holder.

Which action addresses this hazard?

A. Lay it where patients step so it cannot tip from standing height.
B. Ask the next patient to keep a hand on it during the visit.
C. Secure the cylinder in the approved holder using the instructed method.
D. Lean the cylinder against the table without a holder.

Q41 | Phlebotomy

> **SCENARIO** The laboratory label diagram requires the patient label on the tube body, with the barcode flat and the fill-inspection area visible. An assistant proposes placing the label only on the removable cap.

Which labeling method follows the diagram?

A. Wrap it loosely so the barcode wrinkles around the cap and tube.
B. Place it only on a separate rack position rather than on the tube.
C. Place it only on the cap because the cap normally stays attached.
D. Apply the label to the tube body in the specified position without covering the inspection area.

Q42 | General Patient Care

SCENARIO An authorized prescriber gives a verbal medication instruction. The assistant is permitted to receive it under applicable law and clinic procedure, which requires complete read-back. The assistant is uncertain whether the spoken unit was milligrams or micrograms.

What should the assistant do before entering or acting on the order?

A. Clarify the unit directly with the prescriber and read back the complete confirmed order.

B. Use an older order's unit without checking whether the new instruction matches it.

C. Enter the most common unit for this drug and ask for confirmation after administration.

D. Record the numerical dose without clarifying the uncertain spoken unit.

Q43 | General Patient Care

SCENARIO A patient is preparing for a supervised wheelchair-to-table transfer. The wheelchair rolls slightly when the patient shifts forward, and its wheel locks have not been engaged. The planned helpers are present.

What should be corrected before the transfer begins?

A. Begin the transfer quickly before the chair can move again.

B. Engage and verify the wheelchair wheel locks before the patient stands.

C. Ask a helper to hold the backrest while leaving the wheel locks off.

D. Have the patient stand on the footplates to stabilize the chair.

Q44 | Infection Control and Safety

SCENARIO A regulated-waste bag is closed, but blood has contaminated its outside. The clinic plan requires an additional closable, leak-resistant, appropriately labeled container before transport when outside contamination occurs.

Which action follows the plan?

A. Put the bag inside an open paper shopping bag.

B. Carry the bag against a clean uniform so it does not drip.

C. Wipe only the label and transport the bag without the required outer containment.

D. Place the closed contaminated bag in the specified secondary container before transport.

Q45 | Infection Control and Safety

> **SCENARIO** The instructions for a high-level disinfectant require its concentration strip to meet the specified minimum before instruments are processed. Today the strip shows a concentration below that minimum, although the solution's calendar use limit has not been reached.

What should the assistant do?

A. Stop using the bath and replace or manage it according to the product's failed-concentration procedure.
B. Use the bath until its calendar limit regardless of the failed strip.
C. Extend the soak time without replacing the bath or checking a validated correction.
D. Add an estimated amount of concentrate without following a specified preparation process.

Q46 | General Patient Care

> **SCENARIO** A verified medication order schedules an oral dose every 6 hours, with no additional conditions. The last dose was administered at 10:15 a.m. The assistant is checking the schedule, not changing the order.

When is the next scheduled dose due?

A. 4:15 p.m.
B. 2:15 p.m.
C. 10:15 p.m.
D. 6:15 p.m.

Q47 | General Patient Care

> **SCENARIO** An adult begins coughing after a bite of food but can speak clearly and continues a strong, effective cough. The trained assistant follows current clinic first-aid guidance to observe for progression to severe obstruction.

Which action is appropriate while the cough remains effective?

A. Begin chest compressions on the responsive patient who is speaking clearly.
B. Encourage coughing and continue close observation for signs of severe obstruction.
C. Give a drink while the patient is actively coughing.
D. Perform blind finger sweeps to search for food.

Q48 | Point of Care Testing and Laboratory Procedures

SCENARIO A point-of-care test displays a valid positive result under its instructions. The patient asks the assistant whether this confirms a diagnosis and which prescription to start. Clinic workflow requires entering the test result and routing clinical interpretation to the ordering provider.

Which response follows the workflow?

A. Record the result as displayed and arrange provider interpretation and treatment instructions.
B. Select a prescription because the test used a positive label.
C. Change the result to a diagnosis code chosen by the assistant.
D. Withhold the valid result from the provider until the assistant chooses treatment.

Q49 | Patient Intake and Vitals

SCENARIO During history collection, a patient says the patient's mother developed breast cancer at age 43. The patient reports no personal history of breast cancer.

Where should the information be recorded?

A. Surgical history as a procedure already performed on the patient.
B. Personal medical history as the patient's confirmed breast cancer.
C. Medication history as a current anticancer prescription.
D. Family history, identifying the mother and reported age at diagnosis.

Q50 | EKG and Cardiovascular Testing

For a standard 12-lead EKG, where is V3 placed relative to correctly positioned V2 and V4?

A. At the left midaxillary line level with V4.
B. Midway between the V2 and V4 positions.
C. At the left sternal border directly above V2.
D. At the right sternal border level with V2.

Q51 | Infection Control and Safety

SCENARIO A steam sterilizer's biological indicator is reported positive. The clinic response procedure follows the initial steps of removing the sterilizer from service and notifying the designated supervisor and infection-prevention lead.

What should the assistant do about the equipment?

A. Continue routine loads while arranging review because chemical tape changed color.
B. Remove it from routine service and make the required notifications so the investigation can begin.
C. Record only favorable mechanical readings and omit the positive biological result.
D. Rerun the biological monitor but leave the sterilizer in routine service meanwhile.

Q52 | Patient Intake and Vitals

SCENARIO During manual blood pressure measurement on a comfortable patient, an assistant releases cuff pressure so quickly that several millimeter markings pass between heartbeats. The clinic procedure specifies controlled deflation of 2-3 mm Hg per second.

How should the assistant correct the technique?

A. Average the uncertain endpoints with last month's pressure.
B. Repeat immediately with the same fast deflation and select whichever result is lower.
C. Record the rapidly observed sound endpoints without repeating the measurement.
D. Repeat the measurement using the specified controlled deflation rate and required rest interval.

Q53 | General Patient Care

SCENARIO A trained, authorized assistant is carrying out ordered ear irrigation after provider evaluation. The patient suddenly reports sharp pain and dizziness. The clinic procedure requires stopping and notifying the clinician for either symptom.

What should the assistant do?

A. Increase the pressure to clear the obstruction more quickly.
B. Stop the irrigation and promptly report the new symptoms to the clinician.
C. Continue until the planned volume is delivered, then report the symptoms.
D. Tell the patient to stand and walk while irrigation continues.

Q54 | Patient Intake and Vitals

SCENARIO An adult with a communication disability cannot state identifying information. Before a nonurgent measurement, clinic policy permits verification using two approved identifiers from the identification band and a reliable caregiver, matched to the EHR. The caregiver is present, and the band is legible.

Which approach follows this identification procedure?

A. Match the two approved identifiers using the band, caregiver, and EHR before proceeding.
B. Use the caregiver's recognition alone without matching the band or EHR identifiers.
C. Use the appointment slot and room number instead of the approved identifiers.
D. Copy identifiers from the previous patient's open chart to avoid delaying the measurement.

Q55 | Phlebotomy

> **SCENARIO** For a specific capillary collection system, the instructions say to wipe away the first drop with clean dry gauze and collect the next freely forming drop. The puncture has just produced the first drop.

Which action follows these instructions?

A. Wipe the first drop with clean dry gauze and collect the next freely forming drop.
B. Wipe the first drop with a wet surface-disinfectant towel.
C. Collect the first drop regardless of the supplied procedure.
D. Return the wiped blood from the gauze to the collection device.

Q56 | Phlebotomy

> **SCENARIO** A patient has just peeled an orange and has sticky residue on the fingers before an ordered capillary glucose collection. The collection procedure requires washing and thorough drying when food residue is present.

Which preparation is appropriate?

A. Use the juice on the finger to improve the blood drop.
B. Wash with soap and water and dry thoroughly before puncture-site preparation.
C. Collect through the sticky residue because the lancet is sterile.
D. Wipe only the lancet and leave the finger unchanged.

Q57 | Patient Intake and Vitals

> **SCENARIO** During rooming, a patient develops marked breathing difficulty and new confusion. A finger pulse oximeter displays 97%, but the patient looks seriously unwell.

Which response is appropriate?

A. Continue routine intake because the displayed saturation appears normal.
B. Repeat the oximeter until it shows a lower number before notifying clinical staff.
C. Move the sensor and leave the patient alone while the new value stabilizes.
D. Activate the clinic emergency response based on the symptoms while supporting prompt clinical evaluation.

Q58 | Phlebotomy

> **SCENARIO** An order requires EDTA plasma, and the laboratory collection sheet lists EDTA as the only accepted additive for this test. Available tubes are identified by additive rather than cap color.

Which tube should the assistant select?

A. The tube labeled serum clot activator.
B. The tube labeled sodium citrate anticoagulant.
C. The tube labeled EDTA anticoagulant.
D. The plain serum tube with no additive.

Q59 | General Patient Care

> **SCENARIO** A trained, authorized assistant is preparing a vaccine that requires reconstitution with its manufacturer-specified diluent. Only a diluent supplied for a different vaccine is on the tray; the correct one can be obtained.

What should happen before preparation continues?

A. Mix both diluents to approximate the intended composition.
B. Obtain and verify the diluent specified for this vaccine.
C. Use the other vaccine's diluent because both are clear.
D. Substitute stock sterile water without checking the product instructions.

Q60 | General Patient Care

> **SCENARIO** A patient is prepared for examination of a small lesion on the upper arm. The gown and drape allow the rest of the body to remain covered.

Which draping approach is appropriate?

A. Require the patient to hold the gown open with both arms throughout the visit.
B. Expose the needed arm area while keeping unrelated areas covered.
C. Remove all coverings before the clinician enters for easier access.
D. Keep the lesion covered and expose the abdomen instead.

Q61 | General Patient Care

> **SCENARIO** Before discharge, the assistant sees that the current medication list says every 8 hours while the new after-visit instruction says every 12 hours for the same drug. Both appear current, and the patient asks which to follow.

What should the assistant do?

A. Reinforce the medication list without reviewing the contradictory new instruction.
B. Reinforce the newly printed schedule without resolving the conflicting active list.
C. Obtain prescriber clarification and ensure the patient receives one consistent authorized instruction.
D. Give both schedules and ask the patient to choose whichever seems easier.

Q62 | Patient Intake and Vitals

> **SCENARIO** An older adult reports pain of 7 on a 0-10 scale before an ordered intervention and 3 at the provider-specified reassessment time. The EHR has separate baseline and follow-up fields.

Which documentation best preserves the response?

A. Record zero because the score decreased after the intervention.
B. Average the two ratings and record 5 as both assessments.
C. Record both ratings with their times and relevant reported changes.
D. Replace the baseline 7 with 3 so the record contains only the current score.

Q63 | Patient Intake and Vitals

> **SCENARIO** While observing breathing for 60 seconds, an assistant counts 16 chest rises and 16 corresponding falls. Each rise followed by its fall is one complete respiratory cycle.

Which respiratory rate should be documented?

A. 8 breaths/min
B. 32 breaths/min
C. 64 breaths/min
D. 16 breaths/min

Q64 | Infection Control and Safety

> **SCENARIO** A reusable examination sensor must be disinfected between patients. Its manufacturer lists a compatible product and warns that an alcohol wipe damages the sensor material. Both products are available.

Which choice is appropriate?

A. Use the listed compatible product and its full instructed process.
B. Use the alcohol wipe because all clinical sensors tolerate alcohol.
C. Alternate the incompatible product with the compatible one to reduce damage.
D. Skip processing when the sensor surface looks clear.

Q65 | EKG and Cardiovascular Testing

> **SCENARIO** Before a resting EKG, a patient worries that the electrodes will deliver electrical shocks. The assistant is preparing the patient for the test.

Which explanation is accurate?

A. It records the heart's electrical signals without delivering a shock.
B. Each electrode stimulates the heart to produce the tracing.
C. The recorder replaces the heart's natural electrical activity during the test.
D. The test intentionally shocks the chest whenever the patient moves.

Q66 | General Patient Care

> **SCENARIO** A patient has an active order stating nothing by mouth, including medications unless specifically authorized. A previously scheduled oral tablet remains on the task list, but no exception order is present. The patient is stable.

What should the assistant do before administration?

A. Omit this dose permanently and discontinue the oral order independently.
B. Change to an available nonoral formulation without a new order.
C. Give the scheduled tablet with minimal water without obtaining an exception.
D. Hold the oral task and obtain prescriber clarification of the conflicting instructions.

Q67 | Point of Care Testing and Laboratory Procedures

SCENARIO During monocular distance-vision testing, a patient covers the untested eye by pressing the palm firmly against it. The clinic technique requires an occluder without pressure on the eye.

Which correction is appropriate?

A. Use the approved occluder to cover the untested eye without pressure.
B. Record binocular acuity while marking the result as monocular.
C. Ask the patient to press harder to block more light.
D. Have the patient squint both eyes throughout the reading.

Q68 | Infection Control and Safety

SCENARIO The sterilizer instructions for the current packaged load require a single layer of separated packs in the specified rack, with space for circulation. The assistant sees packs tightly stacked on top of one another.

What should be corrected before starting the cycle?

A. Leave the stack and select any longer cycle without checking validation.
B. Load the packs in the specified separated arrangement rather than tightly stacking them.
C. Compress the stack more firmly to prevent shifting.
D. Remove all wrappers so the tightly packed instruments fit better.

Q69 | Phlebotomy

SCENARIO During a venous collection attempt, blood enters with unexpected pulsatile force. The clinic procedure requires stopping unexpected pulsatile-flow collections, applying direct pressure after needle removal, and obtaining prompt clinician review.

What should the assistant do?

A. Continue because forceful flow means the collection will finish faster.
B. Stop, remove the needle safely, apply direct pressure, and report the observation promptly.
C. Label the specimen arterial and consider the event complete without review.
D. Redirect the needle repeatedly while the tourniquet remains tight.

Q70 | Patient Intake and Vitals

SCENARIO A 6-year-old boy's measured height is ready to plot. The clinic procedure specifies the age- and sex-specific height chart for ages 2-20 years. Several pediatric charts are available.

Which chart should the assistant select under this procedure?

A. The girls' weight-for-age chart for ages 2-20 years.
B. An adult BMI chart without an age axis.
C. The boys' height-for-age chart for ages 2-20 years.
D. The infant length-for-age chart for birth through 24 months.

Q71 | Patient Intake and Vitals

> **SCENARIO** A scale display reads 52.4 kg, but the EHR preview imports the number 52.4 into a field labeled lb. The patient remains available, and the value has not been signed.

Which correction is appropriate?

A. Change the source label to pounds without confirming the scale's measurement mode.

B. Accept the import and ask the provider to reinterpret the unit later.

C. Resolve the unit mapping and preserve the measured value with its correct unit before finalizing.

D. Keep 52.4 in the pound field because the numeric display was transcribed correctly.

Q72 | Phlebotomy

> **SCENARIO** A laboratory handling sheet requires specimen A to reach processing within 10 minutes, while specimen B can wait for a routine 2-hour batch. A courier can take specimen A now, but waiting for the batch would exceed its limit.

Which transport decision follows the handling requirements?

A. Release specimen A for prompt processing rather than holding it for the routine batch.

B. Hold both until the routine batch because they were collected in the same visit.

C. Relabel specimen A as specimen B to allow a longer interval.

D. Change specimen A's collection time so it appears to meet the later batch.

Q73 | General Patient Care

Which anatomical area corresponds to the right vastus lateralis muscle?

A. The anterolateral right thigh.

B. The upper outer right buttock.

C. The inner right forearm.

D. The posterior right upper arm.

Q74 | General Patient Care

> **SCENARIO** The provider will examine an adult's ear with an otoscope. The device procedure calls for a clean, appropriately processed speculum that fits comfortably without forcing. Two sizes are available, and the larger one causes discomfort when gently positioned.

Which preparation is appropriate?

A. Use the otoscope without a speculum to avoid choosing a size.

B. Force the larger speculum because a larger opening always improves the examination.

C. Insert the larger speculum farther until the patient stops moving.

D. Select the smaller suitable speculum and report any continuing difficulty to the provider.

Q75 | Infection Control and Safety

> **SCENARIO** An assistant applies alcohol-based hand rub before donning gloves. The hands remain wet, and the product direction is to rub all surfaces until dry.

Which action completes the hand-rub step?

A. Continue rubbing until the hands are dry before putting on gloves.
B. Wipe off the wet product on the uniform sleeve.
C. Put on gloves while the hands remain wet to trap the product.
D. Rinse immediately with water to remove the product before it dries.

Q76 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A specified CLIA-waived test requires its supplied dropper because drop size is part of the instructions. The supplied dropper is available, but a similar-looking dropper from another kit is closer to the assistant.

Which device should be used to deliver the test reagent?

A. An uncalibrated straw cut to a convenient length.
B. The other kit's dropper because both produce visible drops.
C. Any dropper if the assistant doubles the drop count.
D. The dropper supplied and specified for this test system.

Q77 | General Patient Care

> **SCENARIO** A patient who was speaking normally suddenly develops a drooping mouth, one-sided arm weakness, and slurred speech. The assistant witnesses the change at 2:20 p.m. Clinical help is immediately available.

Which response is appropriate?

A. Finish the full history before activating emergency response to determine the likely diagnosis.
B. Notify the provider after the routine intake queue while omitting the witnessed onset time.
C. Activate emergency response immediately and report the witnessed onset time and findings.
D. Ask the patient to wait for a repeat assessment because the symptoms might be temporary.

Q78 | Phlebotomy

> **SCENARIO** The venipuncture site is prepared and dry. To anchor the vein, the clinic technique directs gentle skin traction below the puncture site, with the anchoring hand outside the needle path.

Which technique follows the instruction?

A. Anchor the vein by pinching the needle between bare fingertips.
B. Press a fingertip onto the prepared puncture point while inserting the needle beside it.
C. Place the anchoring thumb directly above the puncture point in the needle path.
D. Apply the instructed traction below the site while keeping the anchoring hand out of the needle path.

Q79 | Patient Intake and Vitals

> **SCENARIO** An assistant palpates a radial pulse with the thumb and notices a second faint pulsation that does not match the patient's beats.

Which technique change is appropriate?

A. Move the thumb to a different point and keep the same technique.
B. Use the finger pads of the index and middle fingers instead of the thumb.
C. Count both pulsations as separate patient beats.
D. Press harder with the thumb until both pulsations disappear.

Q80 | Patient Intake and Vitals

> **SCENARIO** A clinic thermometer has separate probes labeled for oral and rectal use. The manufacturer prohibits interchanging the probes. A rectal-use probe is attached, but an oral temperature is planned.

What should the assistant do?

A. Use the rectal-use probe orally and subtract a fixed temperature difference.
B. Use the rectal-use probe orally with an extra cover.
C. Attach the designated oral-use probe and follow its preparation instructions.
D. Change only the EHR route field while keeping the attached probe.

Q81 | Infection Control and Safety

> **SCENARIO** A reusable pointed instrument is being packaged for steam sterilization. Its packaging instructions require a compatible steam-permeable tip protector that prevents wrapper puncture. An approved protector and several unapproved coverings are available.

Which accessory should the assistant select?

A. The approved steam-compatible, steam-permeable tip protector.
B. A thick improvised covering that blocks steam contact at the tip.
C. Adhesive tape wrapped directly around the point instead of the specified protector.
D. An unapproved airtight plastic cap over the instrument tip.

Q82 | General Patient Care

SCENARIO A liquid chemical splashes into a patient's eye in the clinic. The posted response procedure directs immediate continuous irrigation with the available eyewash while another staff member summons clinical help and obtains the product information. The assistant has current response training.

What should the assistant do?

A. Delay all irrigation until the full incident history is entered into the EHR.
B. Instill a second chemical chosen to neutralize the first one.
C. Begin the directed eyewash immediately while help and product information are obtained.
D. Patch the eye shut and wait for the next routine provider round.

Q83 | General Patient Care

SCENARIO During a minor procedure, the provider places one continuous strip of packing in a wound. The assistant is documenting according to a clinic procedure requiring the number and type of packing pieces left in place.

Which entry best supports later removal?

A. No foreign material present, because the packing is therapeutic.
B. One continuous strip of the specified packing material left in the wound, with the ordered details.
C. Packing removed, although the provider left it in place.
D. Dressing applied, with no description of the material remaining inside the wound.

Q84 | General Patient Care

SCENARIO A trained, authorized assistant is preparing an ordered non-IV injection of 350 mg. The vial contains 1 g of powder. Its instructions state that adding 4.2 mL of the specified diluent produces a final volume of 5 mL; dose calculations must use final volume. A suitable syringe is available.

Which volume provides 350 mg after correct reconstitution?

A. 2.86 mL
B. 1.47 mL
C. 0.35 mL
D. 1.75 mL

ANSWER KEY & EXPLANATIONS

Practice Exam 17 | Domain 3: Clinical Patient Care | 84 scored questions

01 C	02 A	03 C	04 A	05 C	06 B
07 B	08 D	09 B	10 D	11 D	12 A
13 A	14 B	15 A	16 D	17 C	18 B
19 B	20 D	21 B	22 C	23 B	24 B
25 D	26 C	27 B	28 C	29 D	30 A
31 C	32 A	33 C	34 A	35 C	36 B
37 C	38 A	39 D	40 C	41 D	42 A
43 B	44 D	45 A	46 A	47 B	48 A
49 D	50 B	51 B	52 D	53 B	54 A
55 A	56 B	57 D	58 C	59 B	60 B
61 C	62 C	63 D	64 A	65 A	66 D
67 A	68 B	69 B	70 C	71 C	72 A
73 A	74 D	75 A	76 D	77 C	78 D
79 B	80 C	81 A	82 C	83 B	84 D

DETAILED EXPLANATIONS

Q01 | Correct answer: C

Replace the wet mask using the specified removal and hygiene procedure.

The observed condition meets the supplied replacement criterion. Reversing, pocket-drying, or continuing a wet mask does not restore the required fit and condition.

Q02 | Correct answer: A

The uninjured lateral fleshy pad of the ring finger.

The available ring-finger site meets the supplied anatomical and skin-condition criteria. Callused or injured areas and an unlisted palm location do not meet this collection guide.

Q03 | Correct answer: C

Keep the vial patient-dedicated or dispose of it under the stated policy.

The supplied treatment-area rule controls how the vial may subsequently be used. Visual cleanliness of the stopper does not justify returning a patient-area vial to the common supply.

Q04 | Correct answer: A

The age-approved fixed-depth 1.2 mm safety lancet.

The 1.2 mm device is both explicitly age-approved and within the stated 1.5 mm maximum. The deeper or unverified devices do not meet the supplied patient-specific device criteria.

Q05 | Correct answer: C

Follow the interruption procedure and repeat with a new cartridge after restoration.

The interrupted run has not produced a valid reportable result under the supplied instructions. A new cartridge and the specified restored-system procedure are required rather than reuse or estimation.

Q06 | Correct answer: B

Remove them from use and arrange safe disposal and verified replacement under policy.

Unknown syringe contents cannot be made safe by guessing or applying expected labels. The specified identity requirement must be met with a verified replacement rather than inferred from appearance or schedule.

Q07 | Correct answer: B

Release the tourniquet as specified while maintaining safe needle control.

Releasing the tourniquet before withdrawal follows the specified procedure and removes venous pressure from the constricted limb. Tightening it or moving the limb around an inserted needle does not prepare safe withdrawal.

Q08 | Correct answer: D

Record and flag the negative-control finding for clinician review without assigning allergy diagnoses.

The negative-control finding exceeds the explicit review threshold. The assistant should report its effect on test validity to the clinician rather than independently converting the measurements into diagnoses or corrections.

Q09 | Correct answer: B

Elevate the head of the table to the specified range and support the patient safely.

Semi-Fowler positioning in this order is defined by the backrest angle. Raising the entire table does not produce the prescribed head and torso elevation.

Q10 | Correct answer: D

Report the pain and obtain the required clinician review before screening.

The new symptom triggers the supplied review requirement. The assistant should not bypass it by treating an unassessed obstruction or turning a symptom report into a diagnosis.

Q11 | Correct answer: D

Record the actual reported use and flag the difference from the listed directions for clinician review.

The discrepancy should remain visible with the patient's actual reported use. Recording a history is different from independently deciding which dosing instruction the patient should follow.

Q12 | Correct answer: A

Between the cheek and gum.

Buccal placement is between the cheek and gum, as the directions state. Placement under the tongue is sublingual and is a different route.

Q13 | Correct answer: A

1.5 tablets

0.05 mg x 1,000 mcg/mg = 50 mcg per tablet; 75 mcg / 50 mcg per tablet = 1.5 tablets. The unit conversion is required before comparing the order with the tablet strength.

Q14 | Correct answer: B

137.2 cm

(4 x 12) + 6 = 54 in; 54 x 2.54 = 137.16 cm, or 137.2 cm to one decimal place. Omitting the additional 6 in would give 121.9 cm instead.

Q15 | Correct answer: A

0.32 second

8 boxes x 0.04 second per box = 0.32 second. The horizontal time scale must be used; a tenfold decimal shift would produce 3.2 seconds instead.

Q16 | Correct answer: D

Notify the clinician with the actual administration details and follow the directed patient-care response.

An actual administration error requires prompt clinical review even when symptoms are not yet apparent. Accurate details support the response; concealing the error or selecting corrective treatment independently can increase harm.

Q17 | Correct answer: C

Complete the seal and collection entries and document each handoff as required.

The supplied procedure preserves specimen integrity and an accountable transfer record. An unsealed specimen or backdated, delayed entries do not meet those requirements.

Q18 | Correct answer: B

Stop the task safely, remove the damaged gloves, perform hand hygiene, and replace the gloves.

A torn glove no longer provides the intended barrier. The supplied procedure requires safe removal, hand hygiene, and replacement rather than patching or covering the defect.

Q19 | Correct answer: B

Try a new compatible tube as instructed while maintaining safe needle control.

The first tube filled and the second did not despite unchanged needle position, supporting the specified vacuum-troubleshooting step. Replacing the tube avoids unnecessary probing and preserves the collected specimens.

Q20 | Correct answer: D

Complete the remaining required fresh-water rinses before further preparation or use.

The required rinses are separate steps in removing residual disinfectant. Adequate soaking does not eliminate the stated rinsing requirement, and reused water does not meet the fresh-water instruction.

Q21 | Correct answer: B

Open the package without touching its contents and let the gauze fall onto the sterile field.

A controlled drop keeps the nonsterile wrapper and hands off the sterile field. Placing or dragging the outer wrapper onto the field introduces a contamination route.

Q22 | Correct answer: C

Record all three runs and obtain the required review before resuming testing.

The passing run is part of the full history and does not remove the explicit repeated-failure review requirement. Complete documentation lets the reviewer assess whether the problem was resolved rather than hidden by selective retention.

Q23 | Correct answer: B

Keep the right foot clear of the floor during the planned assisted maneuver.

Keeping the right foot clear avoids loading the limb during the planned non-weight-bearing transfer. A brief push or partial toe or heel loading still places weight on that side.

Q24 | Correct answer: B

Stop the draw, safely remove and secure the needle while help supports the patient, and activate the response.

Loss of responsiveness requires patient protection and emergency response while the sharp is controlled safely. Finishing collection or providing oral fluid to an unresponsive patient delays or endangers care.

Q25 | Correct answer: D

Follow the mapped replacement and time-recording instructions; call if the signal remains faulty.

The labeled map and time record preserve the intended electrode configuration and identify the recording gap. Random replacement or deletion of prior data does not follow the supplied recovery process.

Q26 | Correct answer: C

The appropriately marked dedicated oral syringe.

A dedicated oral syringe measures the oral dose and distinguishes its route from injection equipment. Household spoons and unmarked droppers do not provide the required measurement reliability.

Q27 | Correct answer: B

Moisten with sterile saline and wait for the adherent dressing to loosen before removal.

Moistening and allowing the dressing to loosen follows the supplied wound-care method. Forceful dry removal or unapproved substances can damage tissue and do not follow the order.

Q28 | Correct answer: C

Remove the visible soil using the specified precautions, then complete the labeled disinfection process.

Visible soil must be removed before the supplied disinfection process is completed. Merely passing over or covering the soil does not accomplish the specified preparation.

Q29 | Correct answer: D

Relax the abdomen and breathe normally so the reading can be taken at normal exhalation.

The supplied method requires a relaxed abdomen and a normal respiratory phase. Abdominal bracing, deep breath holding, and skin compression change the measurement conditions.

Q30 | Correct answer: A

Remove the faulty cable from use and verify the approved replacement through the equipment process.

The approved comparison isolates an equipment fault rather than a patient finding. Replacing and verifying the compatible cable addresses the demonstrated cause without inventing a rhythm interpretation.

Q31 | Correct answer: C

The approved skin antiseptic appropriate to the procedure.

Product use must match its intended application. A hard-surface disinfectant is not automatically suitable for living tissue and should not replace the approved skin antiseptic.

Q32 | Correct answer: A

Clarify the intended procedure order and reconcile the duplicate through the approved workflow.

Possible duplication needs clarification before care or completion status is recorded. Two visible entries do not establish that the provider intended two procedures, and the assistant should not independently erase the clinical request.

Q33 | Correct answer: C

Identify it as incomplete and follow the review process without assigning a negative answer.

The stated scoring rules do not permit substituting a negative response for missing information. Preserving the incomplete status avoids presenting an unsupported score as valid.

Q34 | Correct answer: A

Pause preparation and have the patient and provider resolve the intended target before proceeding.

A disagreement about the procedural target needs direct clarification before treatment begins. The assistant should not choose a lesion, expand the procedure, or disregard the patient's concern.

Q35 | Correct answer: C

The product manufacturer and lot number linked to each administration.

Manufacturer and lot information link an administration to the affected supply. An anatomical site or pharmacy name does not identify which product lot was used.

Q36 | Correct answer: B

Use a new swab and repeat the specified collection without the unintended contact.

The unintended contact violates the supplied collection method. Wiping or relabeling the swab does not restore the specimen to the required collection conditions.

Q37 | Correct answer: C

Leave the affected area, warn others, and activate the designated trained response.

The supplied plan separates immediate protective actions from cleanup requiring training. An untrained assistant should not improvise handling of the spill or stay in the affected vapor area.

Q38 | Correct answer: A

Gently pull the pinna upward and backward before instilling the ordered drops.

The correct action follows the explicit adult product procedure. Deep insertion of the applicator is unnecessary and risks injury.

Q39 | Correct answer: D

Immediately relay the stop request to the supervising team and follow the test-termination procedure.

The patient's request triggers the supplied test-termination workflow and requires prompt communication. The assistant should not override the request or independently change the exercise protocol.

Q40 | Correct answer: C

Secure the cylinder in the approved holder using the instructed method.

The approved holder prevents unrestrained movement or tipping. A wall, patient, or walking path is not the specified secure storage method.

Q41 | Correct answer: D

Apply the label to the tube body in the specified position without covering the inspection area.

A body label remains with the specimen container when its cap or rack position changes. The supplied orientation also preserves barcode readability and the fill-inspection window.

Q42 | Correct answer: A

Clarify the unit directly with the prescriber and read back the complete confirmed order.

The unit must be confirmed by the authorized prescriber before the order can be reliably recorded or performed. Guessing between milligrams and micrograms can produce a major dose error.

Q43 | Correct answer: B

Engage and verify the wheelchair wheel locks before the patient stands.

Securing the wheelchair prevents rolling during the transfer. The presence of helpers does not eliminate the need to engage and verify the wheel locks.

Q44 | Correct answer: D

Place the closed contaminated bag in the specified secondary container before transport.

The supplied secondary-container requirement addresses contamination on the primary container exterior. An open paper bag or partial wipe does not provide the stated containment.

Q45 | Correct answer: A

Stop using the bath and replace or manage it according to the product's failed-concentration procedure.

Both concentration acceptability and the use limit must be met for the supplied process. A remaining calendar interval does not override a failed required concentration check or justify an unvalidated time change.

Q46 | Correct answer: A

4:15 p.m.

Adding six hours to 10:15 a.m. gives 4:15 p.m. The schedule should follow the actual last administration time and the ordered interval.

Q47 | Correct answer: B

Encourage coughing and continue close observation for signs of severe obstruction.

An effective cough may clear a mild obstruction and should be allowed while the patient is observed. Blind sweeps or treating this presentation as unresponsiveness can cause harm and do not match the described condition.

Q48 | Correct answer: A

Record the result as displayed and arrange provider interpretation and treatment instructions.

The assistant can record and communicate the actual test result while leaving diagnosis and prescribing to the provider. A positive device result is not authorization for an assistant-created treatment plan.

Q49 | Correct answer: D

Family history, identifying the mother and reported age at diagnosis.

The affected relative and age belong in family history. Assigning the relative's condition to the patient would create a false personal diagnosis.

Q50 | Correct answer: B

Midway between the V2 and V4 positions.

V3 lies midway between correctly placed V2 and V4. The left midaxillary position corresponds to a different precordial location and is not the V3 midpoint.

Q51 | Correct answer: B

Remove it from routine service and make the required notifications so the investigation can begin.

The stated initial response addresses a positive biological monitor without assuming that a chemical indicator cancels it. Decisions about investigation, prior loads, and return to service follow the designated review process.

Q52 | Correct answer: D

Repeat the measurement using the specified controlled deflation rate and required rest interval.

Rapid deflation can make the sound endpoints difficult to identify accurately. A properly repeated measurement is preferable to accepting endpoints that were not reliably heard.

Q53 | Correct answer: B

Stop the irrigation and promptly report the new symptoms to the clinician.

The new symptoms meet the explicitly supplied stop criteria. Completion of a planned volume does not take precedence over the required safety response.

Q54 | Correct answer: A

Match the two approved identifiers using the band, caregiver, and EHR before proceeding.

The supplied procedure provides an alternative when verbal self-identification is unavailable. Inability to speak does not make a room number or the caregiver's own identity a valid substitute.

Q55 | Correct answer: A

Wipe the first drop with clean dry gauze and collect the next freely forming drop.

The first-drop instruction is specific to the stated collection system and should be followed. A different product's method or contamination of the drop does not satisfy those instructions.

Q56 | Correct answer: B

Wash with soap and water and dry thoroughly before puncture-site preparation.

Food residue can contaminate a capillary specimen and distort the testing conditions. Washing and drying follow the explicit preparation rule rather than treating a sterile lancet as a substitute for a prepared site.

Q57 | Correct answer: D

Activate the clinic emergency response based on the symptoms while supporting prompt clinical evaluation.

Pulse oximetry does not replace evaluation of serious symptoms and has limitations. A reassuring displayed number must not delay emergency escalation for new confusion and marked breathing difficulty.

Q58 | Correct answer: C

The tube labeled EDTA anticoagulant.

The specified EDTA tube matches both the required specimen and the laboratory's additive requirement. Another anticoagulant is not automatically interchangeable for an assay that accepts only EDTA.

Q59 | Correct answer: B

Obtain and verify the diluent specified for this vaccine.

Vaccine diluents are product-specific and are not interchangeable solely because they look alike. The assistant should obtain the specified diluent rather than substitute another product or stock water.

Q60 | Correct answer: B

Expose the needed arm area while keeping unrelated areas covered.

Draping should provide access to the examination area while preserving coverage elsewhere. Full-body exposure is unnecessary for this localized examination.

Q61 | Correct answer: C

Obtain prescriber clarification and ensure the patient receives one consistent authorized instruction.

Conflicting current instructions require clinical clarification rather than an assistant-selected schedule. Resolving the conflict before release reduces the chance that the patient follows incompatible directions.

Q62 | Correct answer: C

Record both ratings with their times and relevant reported changes.

Separate timed entries preserve the change from baseline to reassessment. Overwriting or averaging the observations loses the actual sequence and patient-reported response.

Q63 | Correct answer: D

16 breaths/min

One inhalation and its corresponding exhalation form one breath. Sixteen complete cycles in 60 seconds represent 16 breaths/min, not 32 separate breaths.

Q64 | Correct answer: A

Use the listed compatible product and its full instructed process.

Disinfection must also be compatible with the device. A familiar product is not an acceptable substitute when the supplied instructions specifically warn against it.

Q65 | Correct answer: A

It records the heart's electrical signals without delivering a shock.

A routine resting EKG records the heart's electrical activity through skin electrodes. It is not a defibrillation or electrical stimulation procedure.

Q66 | Correct answer: D

Hold the oral task and obtain prescriber clarification of the conflicting instructions.

The current restriction explicitly includes medications and conflicts with the pending oral task. A prescriber must resolve the instructions; the assistant should not invent an exception or another route.

Q67 | Correct answer: A

Use the approved occluder to cover the untested eye without pressure.

Covering without pressure follows the supplied testing technique and avoids an unnecessary source of visual disturbance. Pressing, squinting, or changing the test while mislabeling it does not provide the requested monocular measurement.

Q68 | Correct answer: B

Load the packs in the specified separated arrangement rather than tightly stacking them.

The validated loading arrangement is part of the process, not merely a storage preference. An improvised longer cycle or removal of required packaging does not establish that an overloaded arrangement is acceptable.

Q69 | Correct answer: B

Stop, remove the needle safely, apply direct pressure, and report the observation promptly.

The observed flow meets the stated stop criterion and requires the supplied pressure and notification response. The assistant should not convert the event into a completed arterial collection or continue probing.

Q70 | Correct answer: C

The boys' height-for-age chart for ages 2-20 years.

The selected chart matches the measured variable, age range, and sex specified by the clinic procedure. An infant length chart or a weight chart would not display the requested height-for-age information.

Q71 | Correct answer: C

Resolve the unit mapping and preserve the measured value with its correct unit before finalizing.

A matching number with a different unit represents a different mass. The measurement source and EHR mapping must agree before the value is used for care.

Q72 | Correct answer: A

Release specimen A for prompt processing rather than holding it for the routine batch.

The short processing limit determines specimen A's transport priority. A convenient batch schedule does not extend that stated limit or justify changing specimen identification or collection time.

Q73 | Correct answer: A

The anterolateral right thigh.

The vastus lateralis is located in the anterolateral thigh. The other listed areas do not identify the ordered muscle, so selecting one would change the specified site.

Q74 | Correct answer: D

Select the smaller suitable speculum and report any continuing difficulty to the provider.

An appropriately fitting speculum supports the examination without forced insertion. Discomfort with the larger size is a reason to use a suitable smaller option, not to overcome resistance.

Q75 | Correct answer: A

Continue rubbing until the hands are dry before putting on gloves.

Rubbing until dry follows the hand-rub technique and product direction. Wiping, rinsing prematurely, or trapping wet product in gloves does not complete the specified application.

Q76 | Correct answer: D

The dropper supplied and specified for this test system.

The required dropper is part of the supplied method and its reagent volume. Similar appearance does not establish equivalent drop size, and an improvised count change is not validated.

Q77 | Correct answer: C

Activate emergency response immediately and report the witnessed onset time and findings.

Sudden focal weakness and speech changes require immediate emergency escalation. Reporting the actual onset time supports timely clinical decisions without requiring the assistant to diagnose the cause or select treatment.

Q78 | Correct answer: D

Apply the instructed traction below the site while keeping the anchoring hand out of the needle path.

The supplied anchoring method stabilizes the area without placing the hand in the advancing needle's path. Contact at the prepared puncture point also creates a contamination risk.

Q79 | Correct answer: B

Use the finger pads of the index and middle fingers instead of the thumb.

The thumb has its own palpable pulse and can confuse pulse counting. Using appropriate finger pads avoids introducing that source of error.

Q80 | Correct answer: C

Attach the designated oral-use probe and follow its preparation instructions.

The designated oral probe follows the explicit device instruction. A new cover or numerical correction does not authorize interchanging probes that the manufacturer separates by route.

Q81 | Correct answer: A

The approved steam-compatible, steam-permeable tip protector.

The specified protector addresses wrapper puncture while preserving the validated process at the instrument tip. An improvised covering may block sterilant contact or fail the packaging instructions.

Q82 | Correct answer: C

Begin the directed eyewash immediately while help and product information are obtained.

The supplied procedure prioritizes immediate irrigation while other response tasks occur in parallel. Waiting for documentation or attempting chemical neutralization does not follow that emergency instruction.

Q83 | Correct answer: B

One continuous strip of the specified packing material left in the wound, with the ordered details.

Recording the material and number of pieces remaining supports subsequent removal and continuity of care. A general dressing entry does not identify what was intentionally left within the wound.

Q84 | Correct answer: D

1.75 mL

1 g = 1,000 mg; 1,000 mg / 5 mL = 200 mg/mL, and 350 mg / 200 mg/mL = 1.75 mL. Using the 4.2 mL added diluent instead of the specified 5 mL final volume gives an incorrect concentration.

End of Practice Exam